

Question:

A mental health volunteer is interviewing locals as part of a community outreach program. A 46-year-old man discloses that he has felt sad for as long as he can remember. He feels as though his life is cursed and if something terrible can happen to him, it usually will. He has difficulty making decisions and feels hopeless. He also feels that he has had worsening suicidal ideations, guilt from past problems, energy, and concentration over the past 2 weeks. He is otherwise getting enough sleep and able to hold a job. Which of the following statement best describes this patient's condition?

Answer Choices:

- A. The patient is at risk for double depression.
- B. The patient may have symptoms of mania or psychosis.
- C. The patient is likely to show anhedonia.
- D. The patient likely has paranoid personality disorder.

Reasoning + Answer:

<think>

Finding Reasoning Paths :

- 1 . **Excessive guilt**: Excessive or pervasive guilt could be linked to depressive disorders such as major depressive disorder (MDD) or persistent depressive disorder (PDD).
- 2 . **Diminished concentration**: Diminished concentration may indicate cognitive symptoms of depression , such as attention or executive dysfunction .
- 3 . **Feeling hopeless**: Hopelessness is a hallmark symptom of depression , often associated with severe depressive episodes or chronic depressive disorders .
- 4 . **Suicidal ideation**: Suicidal ideation , particularly if recurrent , is a strong indicator of a depressive disorder or persistent depressive tendencies .
- 5 . **Sleep and energy**: The patient reports no sleep disturbance or changes in energy , which may suggest a chronic or dysthymic depression rather than a severe depressive episode with somatic symptoms .
- 6 . **Anhedonia**: The description suggests an inability to feel pleasure or interest (anhedonia), which is a core symptom of depression .

Reasoning Process :

- 1 . **Key Symptoms** :
 - The patient reports pervasive sadness , hopelessness , suicidal ideation , guilt , and diminished concentration . These symptoms align with a depressive disorder .
 - The absence of significant sleep disturbances and the description of " getting enough sleep " suggest a non - acute depressive condition rather than a severe depressive episode with hypersomnia or insomnia .
 - The chronic nature of his symptoms (lasting as long as he can remember) supports a diagnosis of a persistent or chronic depressive disorder .
- 2 . **Excessive Guilt** :
 - Excessive guilt is a common symptom in depressive disorders , particularly persistent depressive disorder (PDD) or major depressive disorder (MDD). However , the patient 's description of feeling this way for as long as he can remember aligns more closely with persistent depressive disorder .
- 3 . **Diminished Concentration** :
 - Diminished concentration is a cognitive symptom of depression . While it can be seen in acute episodes of MDD , the chronic nature of the symptoms and the absence of other acute somatic symptoms (e.g ., insomnia or hypersomnia) suggest a more chronic condition .
- 4 . **Feeling Hopeless and Suicidal Ideation** :
 - Hopelessness and suicidal ideation are severe symptoms of depression . These symptoms , combined with the chronicity of the illness , further support a diagnosis of persistent depressive disorder (PDD), which is characterized by recurrent depressive episodes and chronic low mood .
- 5 . **Anhedonia** :
 - Anhedonia , or the inability to feel pleasure , is a hallmark symptom of depression . It is often present in both acute and chronic depressive conditions , including PDD .
- 6 . **Differential Diagnosis** :
 - **Major Depressive Disorder (MDD)**: While the patient’s symptoms overlap with MDD , the chronicity of his symptoms and the absence of significant somatic symptoms (e.g ., sleep changes) make PDD more likely .
 - **Persistent Depressive Disorder (PDD)**: This condition is characterized by chronic low mood , anhedonia , excessive guilt , and suicidal ideation , often without acute episodes of severe depression .
- 7 . **Treatment Considerations** :
 - Persistent depressive disorder is often treated with antidepressants (e.g ., SSRIs) or psychotherapy . Cognitive-behavioral therapy (CBT) is particularly effective for addressing hopelessness and suicidal ideation .
 - Mirtazapine , an antidepressant with sedative and appetite-stimulating properties , could be considered if needed to address somatic symptoms . However , the patient’s lack of sleep disturbance and normal energy levels make this less likely to be the primary concern .

Conclusion :

The patient is likely experiencing **persistent depressive disorder (PDD)**, characterized by an inability to feel pleasure (anh